SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Durvalumab 1500 mg every 28 days

Stage III NSCLC · consolidation after concurrent chemoradiotherapy · NCCP 00655 · version 3a · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Durvalumab is given after definitive chemoradiotherapy to reduce the chance of lung cancer returning or worsening and improve the chance of longer survival. This is part of a treatment pathway that aims for long-term control and possible cure; it cannot guarantee cure.

Schedule
Durvalumab is infused on Day 1 of each 28-day cycle. The course lasts up to 12 months (13 cycles), and can finish earlier if the cancer progresses or side effects make continuation unsuitable. The first dose should start within 42 days after chemoradiotherapy finishes.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Durvalumab

PD-L1 IMMUNE CHECKPOINT INHIBITOR

- ☐ Inflammation of the lungs, bowel, liver, kidneys or skin
- ☐ Thyroid, adrenal, pituitary or blood-sugar problems that may need long-term replacement treatment
- ☐ Rare serious inflammation of the heart, muscles or nerves
- ☐ Infusion reactions, fatigue and infections; effects can appear after treatment ends

Discuss the consolidation aim and possible benefit after chemoradiotherapy, alternatives and the option to decline; lung inflammation after chest radiation; immune inflammation of other organs; possible long-term hormone replacement; infusion reactions; reproductive precautions; tests, visits, the course cap and possible holds or permanent stopping. Trial results do not guarantee cure or individual benefit.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00655 v3a.

SACTCheck v0.84.26
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